14.7 Chest Trauma (Rib Fractures and Flail Chest)

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I cannot take a deep breath because of pain." **Objective** • One side of the chest moves the wrong way (paradoxical) • RR 30/min and shallow • SpO_2 89%	**ACTUAL** **Ineffective breathing pattern** related to chest wall instability and pain as evidenced by paradoxical movement, tachypnea and hypoxemia.	Within 12 hours, RR 12–24/min and SpO_2 94% or more.	• Give oxygen as prescribed. • Sit him upright, or on the injured side if ordered. • Make sure analgesia works well enough (nerve block or epidural as ordered) for him to breathe deeply. • Follow SpO_2, RR and ABG. • If respiratory failure develops, prepare for ventilation.	Breathing effectively: **goal met**.
Subjective • Pain 8/10 when he breathes **Objective** • Ribs tender • Holds his chest stiff	**ACTUAL** **Acute pain** related to rib fractures as evidenced by pain on breathing and splinting.	Pain 4/10 or less when he breathes deeply.	• Give analgesia regularly. • Teach him to hold a pillow against his chest when he coughs. • Check pain before and after breathing exercises.	Pain controlled: **goal met**.
Subjective • *No specific complaint* **Objective** • Breathing shallow • Secretions building up	**RISK** **Risk for infection** as evidenced by risk factors: hypoventilation and retained secretions (pneumonia).	No chest infection.	• While awake, encourage the incentive spirometer every hour. • Encourage coughing once the analgesia is working. • Watch his temperature and sputum.	No infection: **goal met**.